

HOSPITAL SERIAN

TITLE: POLICY FOR REUSE OF DISPOSABLE ITEM

DOCUMENT IDENTIFICATION:

DOCUMENT NO: HS/P&P/05 Prevention and Control of Infection (Supplementary)

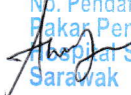
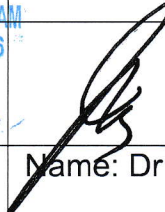
UNIT/DEPARTMENT/DIVISION: Prevention and Control of Infection of HOSPITAL SERIAN

EFFECTIVE DATE: 2th April 2024

NEXT REVIEW DATE: 2th April 2027

VERSION: 2

REVISION: 1

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AMENDMENT PAGE

Date of Amendment	Issues No / Amendment No.	Details of Amendment	Signature of Person who initiated the change	Signature of Person who approved the change
2 th April 2024	2	Added details of the process and itemized the list of reprocessing	Infection control	

CONTROLLED DOCUMENT LIST

Controlled Copy No.	Recipient
1	Prevention and Control of Infection
2	CSSD
3	Quality Unit
4	Pengarah Office

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POLICY FOR REUSE OF DISPOSABLE ITEM

General Guidelines

1. Ideally all disposable or single use items shall not be reused by far possible.
2. The rational of allowing items to be reused was due to the limited supply and resource limited setting.
3. All the items stated below must undergo strict disinfection process and kept clean before each use to negate the risk of cross infection.
4. Maintain good record of patients (including name, diagnosis, date) for each Non-Invasive or Invasive Ventilator use or LMA.
5. Maintain good record of checking system of each set. Label the date of cleaning completed and ready to use.
6. Each clinical area needs to prepare 2 containers for this purpose, one for dirty and another one for clean. Do not use sterile container for clean items, it is only for sterile items.
7. Strictly NOT for reuse following the usage in the patient with a known transmittable disease (PTB, COVID, Influenzae, Varicella Pneumonitis etc). Throw away the set after being exposed to these highly infectious cases regardless of how many times it was used.
8. For infectious cases, to use the item with highest number recorded of reuse, discard immediately after use.
9. In the case of new set being used for infectious cases, it should be disposed immediately after use.
10. Cleaning of item must be thorough and follow the recommended work of process then kept clean and ready for next patient's use.
11. Decision for reuse:
 - a. Appear in good condition upon inspection and checking.
 - b. No signs of dysfunctional of its original purpose.
 - c. Not more than 5 times of use.
12. Definition of wear and tear, and should NOT be reused:
 - a. Appear not in good condition upon inspection and checking.
 - b. Dirt that cannot be removed.
 - c. Signs of dysfunctional of its original purpose, such as leak, crack or distorted.
 - d. Discoloured parts.
 - e. More than 5 times of use.
 - f. After being expose to transmittable disease as stated above.

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Disposable but reuse items, cleaning & disinfection process, frequency of use as below.

	ITEM	Cleaning Process	Remark
1	Guedel airway, Ambu-bag, Stylet	Standard Soaking	Until wear & tear
2	Kidney dish or tray, non-heat resistant	Standard Soaking	Until wear & tear
3	Oxygen tubing (connecting oxygen source to ambubag or CPAP/BiPAP machine)	Standard Soaking	Until wear & tear
4	Oronasal or Nasal mask for CPAP or BiPAP or DuoPAP	Standard Soaking	Until wear & tear
5	Nasal prong patient interface for high flow nasal cannula (Optic flow, AIRVO, AirBlend, etc)	Standard Soaking	Until wear & tear
6	T-connector for nebuliser or MDI	Standard Soaking	Until wear & tear
7	Neopuff tubing and set	Standard Soaking	Until max up to 5x
8	Laryngeal Mask Airway (LMA)	Standard Soaking	Until max up to 5x
9	Breathing Circuit (Single Limb) for Non-Invasive Ventilation (NIV) [such as CPAP or BiPAP or DuoPAP]	Standard Soaking	Until max up to 5x
10	Breathing Circuit (Dual Limb) for Invasive Ventilation	Standard Soaking	Until max up to 5x
11	Nebulizer apparatus (Facemask , nebulizer reservoir, tubing)	Standard Soaking	Until max up to 5x
12	Breathing Circuit with heated wire	Only wipe with non-alcohol disinfectant wipe	Soaking will spoil the heated wire. Until max up to 5x

Those in bold are using high level disinfectant & sterilant agent, such as EndoPA.

Items that to be processed and cleaned at respective units.

	ITEM	Cleaning Process	Remark
1	Head gear for holding patient interface	Wash and Dry	Usual linen Until wear & tear
2	Dialyzer high flux set for non-Hepatitis HD patient	Mesin Repressor	Max up to 12 times. Reprocessing guideline at unit HDU

General rinsing is done by staff at relevant clinical area

Standard soaking is done by CSSD staff at CSSD cleaning area

Drying processing is carried out by CSSD staff.

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Model of Good Care of Manual Cleaning and Disinfection

No	Process	Criteria	Std	Responsibility
1	Remove from patient or equipment	Disconnect item or set completely from patient	100%	SN / JM
2	General Rinsing	Rinse and remove all visible soiling such as blood, mucus or secretion. Except breathing circuit with heated wire, only for thorough wipe	100%	JM
3	Record in dispatch book	Record the items sent and dated	100%	PPK
4	Send to CSSD	Place in dirty container (labeled location of the clinical area user) and send to CSSD during CSSD operating hour	100%	PPK
5	Full personal protective equipment (PPE)	Put on appropriate PPE, e.g: mask, apron, glove, and face shield.	100%	CSSD staff
6	Correctly prepare detergent for soaking	Use appropriate detergent as per policy and follow dilution as recommended.	100%	
7	Dismantle the items	All parts are handled with care and make complete	100%	
8	Thorough cleaning in and out	Wash with soft span/brush under water level to clean thoroughly	100%	
9	Soaking the items	Adequately soaked in appropriate detergent Soaking duration: 5 or 15 minutes	100%	
10	Check and rinse	Until water is clear and clean.	100%	
11	Lay items at clean linen	To dry outer surface of the items and no more water dripping	100%	
12	Transfer item to drying cabinet	Arrange items in drying cabinet according to the recommendation.	100%	
13	Choose Program for drying process	Fully dry the item e.g tubing, ventilator accessories and ambubag in dryer.	100%	
14	Check items that is fully dry	After the selected program cycle completed, check the items one by one for complete dryness.	100%	
15	Transfer item to clean container	Remove the fully dried items from drying cabinet to clean container	100%	PPK
16	Inform ward to collect	Collect items from CSSD	100%	
17	Packing and label	Pack in new clean plastic bag, label date of cleaning and use permanent marker to mark the number of usage (drawing one short stroke) at the item of each part of the set. Label location of the clinical area user (if applicable).	100%	JM
18	Storage and ready for next use	Store at cabinet or emergency trolley accordingly	100%	JM

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CLEANSING DETERGENT AND DRYING CABINET IN CSSU

1. Detergent dilution

There are two (2) types of manual disinfection process, namely high-level disinfection and ordinary disinfection. Hence, CSSD is using two (2) different disinfectants.

- a) **High level disinfection & sterilant:** Using **EndoPA** for Endoscopes & Medical Instruments and respiratory patient interface apparatus such as LMA, oronasal or nasal mask, high flow nasal prong or nasal cannula for infant, child or adult, ambubag, Guedel airway, stylet, respiratory device that direct contact with patient secretion, and etc.

	Detergent	Dilution	Remark
1	EndoPA	5L Peracetic Acid 0.2 % Add Activator 112.5ml (Acetate Sodium Hydrate contains Hydrogen Peroxide) Mix and gently agitate the mixture become activated working solution . * Keep for few minutes before use for soaking	Use container of between 10L and 20L for immersion of instruments or apparatus. Contact time: 5 minutes for high level disinfection 10 minutes for sterilization Rinse before sending for drying. Re-usable up to 14 days <u>or</u> 80 cycles of immersions, whichever comes first. Working solution efficacy can be tested using test strip.

- b) **Ordinary disinfection:** Using Septo PreClean for manual cleaning & disinfecting.

The volume of container for soaking in CSSD is either 30L or 50L.

	Type of Detergent	Method
1	Deconex 53 at 2.0%	Dilute 20ml Deconex 53 with 980mls water Dilute 1L Deconex 53 with 49L water
2	neodisher® Septo PreClean at 1.5% (use up to 7 days)	Dilute 15ml neodisher® Septo PreClean with 985mls water Dilute 450ml neodisher® Septo PreClean with 29.55L water Dilute 750ml neodisher® Septo PreClean with 49.25L water

2. Drying Cabinet Program

There are 3 programs preset in Drying Cabinet.

The principle of drying is to dry until the items are fully dried, sometimes requires up to 2 or 3 cycles depends on the program and the structure and characteristics of the items.

Program	Temperature	Duration	Remark
P1	50 degrees	4-6 hours	General Items; Heated wire breathing circuit
P2	60 degrees	4-6 hours	General items
P3	70 degrees	4-6 hours	More heat-resistant items

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Workflow for cleaning process of reuse of disposable items

